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Transitions of Care Description

The Transitions of Care guidelines are designed to assess a patient's transition of care needs, provide evidence-based care planning, and promote condition self-management for patients with post-acute care needs. The Transitions of Care guidelines help healthcare professionals standardize and improve care coordination assessment and planning during the transition between care settings, while providing resource coordination, education, and advocacy.

The Condition Self-Management guidelines provide a stratified approach to managing patients in transition. These guidelines are condition-specific and are for situations in which patients are likely to benefit from case management to ensure they understand their care needs and that these needs are met appropriately. The condition-specific guidelines are meant to support the population managed telephonically or in-person with full assessments and plan-of-care components. Each condition covered contains a comprehensive set of patient education materials. These materials are written in layperson language (fourth-grade to sixth-grade reading level) and may be sent or given directly to patients.

Transitions of Care Overview

The purpose of the Transitions of Care guidelines is to provide a supporting framework for programs that work to improve the transitions between care settings, prevent avoidable hospital readmissions, and promote patient self-management. Transitions of Care provides standardization of care planning for post-hospital and post-acute care transitions.

Transitions of Care Title and Code

Transitions of Care guidelines can be accessed directly from the table of contents page or via Search.

For identification purposes, each guideline is assigned a "Guideline Code," for example, TC-0003. In this instance, the "TC" designates Transitions of Care. The numbers that follow the prefix are unique to this guideline, which is Asthma.

Transitions of Care Guidelines

A Transitions of Care guideline is comprised of assessment questions with answer selections and corresponding barriers, problems, goals, and interventions. Users can combine the guidelines to create a comprehensive assessment and care plan that focuses on the various elements of optimal patient care. For example, users may elect to include guidelines for preoperative care coordination, home care coordination, outpatient condition self-management, and home safety evaluation.

Assessment Sets

There are 7 assessment sets included in Transitions of Care: Enrollment, Pre-Hospital Planning, Hospital Transition Planning, Post-Acute Care Transition Planning, Outpatient, General, and Case Closure.

Guideline Types

Assessment sets include the following types of Transitions of Care guidelines to help users identify the scope or audience for a particular assessment: assessment and planning, record reviews, patient interviews, case management evaluations, level of care assessment tools, checklists, and follow-up. The assessment and planning guidelines address caregiver support and patient understanding of tasks to be completed (eg, evaluation or testing, medicine changes, level of care determination, or healthcare costs) and look for problems the patient may have with completing these tasks. Record review guidelines are intended to collect relevant transition planning information from available medical record sources. They help to ascertain the patient's location prior to admission, the status of advance care planning documents, and available healthcare coverage. Patient interviews are meant to be done during the direct patient (or appropriate caregiver) interaction and identify the patient's transition preferences, safety issues, knowledge, and psychosocial needs. Case management evaluations help the case manager identify transition needs, options, and issues to create safe transition plans. Level of care assessment tools address admission to home care or skilled nursing care and align with Centers for Medicare and Medicaid Services' patient assessment instruments. The checklists are simplified guidelines to quickly identify problems with the functional elements of a transition plan. Follow-up guidelines help the user evaluate the patient's status, treatment plan, and understanding of follow-up activities (eg, follow-up appointments, medication changes, pharmacy/medical supplies or equipment, new therapies ordered, or laboratory work) and looks for problems the patient may have with completing these activities.

Enrollment

Three guidelines make up the Enrollment section: Evaluation for Case Management, Case Management Program Enrollment, and the Case Management Start of Care Assessment. The Evaluation for Case Management is a high-level guideline that helps users determine which patients are likely to need case management or care coordination support. The Case Management Program Enrollment guideline helps users establish eligibility for the case management program. It also provides information about case management that can be shared with the patient so they can make an informed decision about whether or not to enter the program. The Case Management Start of Care Assessment is a comprehensive evaluation of the patient's medical, behavioral, and psychosocial status, intended to help identify the patient's needs and prioritize care.

Social Determinants of Health

The Social Determinants of Health guidelines are assessments that can help identify the patient's needs and prioritize further assessment and care plan needs. These guidelines are comprehensive, covering elements of social, psychosocial, economic, and physical needs. This section can help healthcare professionals determine if the patient has access to sufficient caregiver, community, and financial support. Assessment and care-plan elements are provided to identify barriers (such as lack of health literacy) or social issues (such as transportation issues) that are adversely affecting the patient's ability to maintain optimal health. The Social Determinants of Health Screening Assessment included in this section provides a baseline to determine if the patient is experiencing food insecurity, housing concerns, or safety issues and is a starting point to build a plan of care that incorporates effective interventions and referrals. Resources and suggestions for referrals are provided at the end of the assessment and may vary by region.

Pre-Hospital Planning

When patients have a planned or elective procedure, proper planning can help ensure that the procedure is not delayed and that if post-procedure care is needed, that care is arranged according to patient preferences and early enough to prevent a delay in discharge. The Preoperative Care Coordination and Outpatient or Short-Stay Surgery guidelines are comprehensive assessments that can help identify the patient's needs and prioritize the assessments and care plans to ensure that all preoperative, outpatient, or short-stay care needs are met and transition plans are initiated. The Perioperative Care Coordination Patient Interview identifies the patient's knowledge of the procedure and psychosocial needs.

Hospital Transition Planning

The Hospital Transition Planning section has 3 major subsections: Initial, Concurrent, and Transition Coordination. The Hospital Transition Planning section provides assessments and care plan activities or interventions that the patient may need to ensure a timely, complete, and safe transition after acute care.

Initial

The Initial section is comprised of a Record Review and Patient Interview guideline. The user completes these guidelines as the initial step toward transitioning the patient to another level of care.

Concurrent

The Concurrent section is comprised of guidelines to support the preparation of a care transition plan and assist the user in evaluating how imminent the discharge may be. The Hospital Care Progression - Assessment and Planning guideline may help the user evaluate how well the patient is progressing toward discharge by identifying care needs, evaluating for potential discharge problems, and taking actions to ensure the patient gets needed care and initiate the transition plan. The Hospital Transition Management - Assessment and Planning guideline assists the user in collating information from the care team and determining primary and secondary transition plans that correspond to patient preferences, care team recommendations, and available resources.

Transition Coordination

The Transition Coordination section contains guidelines to support the logistical aspects of patient transitions between care settings and to support follow-up contact with patients to help identify potential care gaps. The Assessment and Planning guidelines in this section titled Post-Acute Skilled Care Facility Coordination, Home Care Coordination, Long-Term Care Facility Coordination, and Behavioral Health Coordination help the user take steps to facilitate a transfer to a care provider at a lower level of care and resolve issues that may cause transition problems, such as lack of benefits or insufficient provider availability. The Hospital Transition - Checklist provides the user with a complete checklist to ensure that discharge and transition activities have been completed, with corresponding care plan activities or interventions to address incomplete items. The Follow-Up guidelines help the user evaluate the patient's status (eg, follow-up appointments, medication changes, and new therapies ordered) after an emergency department visit, transition to home, or transition to post-acute services.

Post-Acute Care Transition Planning

The Post-Acute Care Transition Planning section has 3 major subsections: Initial, Concurrent, and Transition Coordination. The Post-Acute Care Transition Planning section provides assessments and care plan activities or interventions the patient may need to ensure a timely, complete, and safe transition after care with a post-acute provider.

Outpatient

The Outpatient section contains guidelines for case management activities when the patient transitions to community-based care (such as to their home with outpatient follow-up).

Care Coordination Evaluation - Record Review: This guideline helps users collect key information from existing medical records to assess the patient's status, identify needs, and reconcile conflicts with the patient's self-management plan.

Condition Self-Management: This section is comprised of condition-specific guidelines. Each guideline starts with an assessment that presents questions to identify what the patient knows about their condition, the ordered treatments, and the self-management plan. The assessment then guides the creation of a plan of care while supporting the patient's own self-management. Each guideline provides patient education interventions that help ensure the patient knows how to monitor for problems due to the condition, has an action plan should the condition worsen, and knows how to prevent the condition from causing problems (eg, by taking medication or following a diet).

The assessment portion of the Condition Self-Management guidelines is broken into sections titled Patient Interview and Case Management Evaluation to facilitate the use of the guidelines at multiple points of engagement. The Patient Interview section is broken into subsections to allow users to focus on a particular aspect of the patient's self-management activities when they are engaging directly with the patient. These are:

- **Self-Care:** evaluates patient understanding of the activities the patient needs to do to monitor and manage their condition, such as blood pressure checks or weight monitoring
- **Activity:** evaluates what the patient knows about the activity appropriate for the condition and identifies potential issues affecting the patient's ability to stay active and exercise
- **Nutrition:** evaluates what dietary plans the patient is following
- **Medicine:** facilitates review of medications the patient may be taking for a particular condition and identifies problems the patient may have related to medication use or side effects
- **Complications:** identifies patient understanding or existence of potential complications and covers issues such as symptoms of diagnosis and what to do if symptoms occur
- **Follow-Up Care:** evaluates the plan and patient understanding of follow-up care activities (such as procedures, appointments, or laboratory work) and looks for problems the patient may have with completing these activities

In the Case Management Evaluation section, the case manager can then review the patient's knowledge and reconcile it with the provider's treatment plan, thereby identifying care gaps or conflicts as well as patient education needs.

Provider Appointment - Follow-Up and Urgent Care - Follow-Up: These guidelines help the user evaluate changes in the patient's care plan, along with the patient's understanding of these changes, after a recent provider appointment or urgent care visit.

General

The General section helps clinicians identify issues such as social determinants of health, home safety, medication reconciliation, hospitalization risk evaluation, and readiness to change. The guidelines are intended to be used at any appropriate point in the patient's care, such as after a doctor's appointment or following a hospital stay.

Case Closure

The Transitions of Care Case Management Closure guideline contains a brief assessment to help case managers document why a case is being considered for closure, as well as a plan of care to identify problems, goals, and interventions.

Transitions of Care Format

The interactive format (eg, Indicia for Care Coordination) makes it easier for healthcare professionals to move quickly through the assessment portion of guidelines and formulate the most appropriate goals and identify optimal interventions. Depending on how the user answers each question, the software automatically culls through the remaining questions and presents only those that are relevant. Once the assessment is completed, only those goals and interventions that address the problems identified are displayed. The interactive guidelines are fully documentable. In the static format, all questions, answers, barriers, problems, goals, and interventions are displayed where relevant in each Transitions of Care guideline.

Assessment

Many questions are written to solicit information from the patient (ie, are open-ended), using the principles of motivational interviewing, which allows the user to evaluate what the patient already knows vs leading the patient to a particular answer. The questions aim to identify problems so that appropriate goals and interventions can be established as well as to collect data for outcomes reporting. The answers include common answers that patients may provide on a particular subject. An "other" answer is available in multiple-choice questions to provide a place to document when an answer selection is not available.

There are 3 audience types in the Transitions of Care guidelines to help users identify the person to whom the question is directed. If the question is intended to be asked of the patient directly, the question will have the delineation "Patient." The question will be delineated "Caregiver" when it should be asked of the appropriately designated family member or other person caring for the patient. This becomes important when the patient potentially has impairments that require support for their self-management. Case management-related assessment questions are tagged "Case Manager." These questions are intended for the case manager to evaluate the information collected and to make evaluations on next steps or priorities for care planning activities. The audience-type designation is for user reference only and not intended to be viewed by the patient or caregiver. As different users refer to their audience in multiple ways (eg, member, beneficiary, client), "Patient" is intended for identification of the audience only.

Plan of Care

The Plan of Care identifies barriers, problems, goals, and interventions that can help clinicians optimize the patient's care plan.

Barriers

Barriers are issues that would prevent the patient from participating in the program and that should be addressed before continuing with further assessment and care planning. Individualized barriers to goals are identified by answers to assessment questions. Barriers triggered by patient answers should be resolved before other care plan priorities are implemented. Barriers in content are often related to psychosocial needs and, until addressed, implementing a plan of care might not be effective. Consider referencing the Social Determinants of Health Screening Assessment as a first step in screening for social determinants of health needs. The identification of barriers provides the clinician with the information required to select the interventions needed to achieve the goals set by the patient and clinician.

Problems

Problems are generated when the answer to a particular question or set of questions identifies patient care needs. Problems include the need for more information about the condition or establishment of a self-management plan.

Goals

Goals are desired outcomes. They address deficiencies identified in the assessment. Goals include improved knowledge of the condition, better self-monitoring and management of the condition, effective medication management, and tobacco and alcohol abstinence, among many others.

Interventions

Interventions list the specific strategies and actions that healthcare professionals can take to help patients meet the identified goals. These actions fall into 5 categories of "Reconcile," "Assist," "Educate," "Coordinate," and "Send," which delineate the type of action the healthcare professional will perform. Reconcile interventions may prompt the clinicians to perform an activity during the plan of care (such as when a conflict with an order exists or resources are not available). Assist interventions are those the clinician will assist the patient in doing, with the patient leading the action. Educate interventions are the items the clinician can provide to educate the patient. Coordinate interventions are those the clinician will assist the patient in doing, with the nurse leading the action. Send interventions identify related items that the patient should be given. Clinicians may be prompted to perform interventions from several or all of these categories. For example, clinicians may be prompted to reconcile a medication list, assist the patient in obtaining a physician evaluation, educate the patient about symptoms to watch for, coordinate referrals to other healthcare providers, and send appropriate patient education materials.

Transitions of Care Evidence Summary

The Evidence Summary presents discussion and review of the evidence that supports the statements made in the guideline. The Evidence Summary summarizes the results of studies that also support a particular care plan activity or intervention, such as what treatments have been shown to have the greatest effect for patients with a particular condition or what interventions (such as tobacco cessation and exercise) can have a beneficial effect on wellness. The reviews, trials, series, reports, and analyses that form the evidence base are cited, summarized, and interpreted throughout the Evidence Summary and serve as the foundation for best practice recommendations for delivery of case management.

Transitions of Care Patient Education

The Transitions of Care guidelines provide educational materials written at a fourth-grade to sixth-grade reading level that can be given to the patient. Readability level is gauged using several readability tools, including Spache, Dale-Chall, Flesch-Kincaid, SMOG, and others. The patient handouts are available in PDF for easy printing and also in Spanish. Handouts include several diaries, such as a weight diary, that can assist patients in tracking their progress against goals. The evidence supporting the Patient Education materials may be found cited within the Plan of Care and Evidence Summaries of the Transitions of Care guidelines.

Patient Education handouts are developed and designed to address the fact that patients have varying literacy skills through word choice, sentence construction, layout and design, and visual aids. The handouts are written in the active voice and in a positive tone, using second-person pronouns (you/your). Sentences are short and simple, with minimal use of compound or complex sentences. Words with 1 or 2 syllables are used whenever possible. Some exceptions include multisyllable medical and technical terms when they cannot be substituted with another word (eg, "oxygen," "glucometer"). The layout and design follow principles intended to make content easier to read and understand, such as using large amounts of white space, breaking content into short sections with simple headings, presenting complex ideas in easy-to-follow steps and lists, and using a font that is 12 point in size or larger. Many handouts contain icons to help readers understand the main topics of particular sections, such as a medicine bottle icon next to the header "Medicines" and knife and fork next to "Eating Plan." There are also illustrated handouts with step-by-step pictures that walk the patient through a particular care process or activity.

Benchmarks and Data

Benchmarks and Data for Transitions of Care provides information that organizations may use to compare their performance to that of other organizations based on a nationwide data sample. This includes condition-specific information on length of stay by level of care, number of home care visits, percentage of patients discharged from a hospital to a post-acute level of care (ie, recovery facility or home care), and hospital readmission rates.

Benchmark Statistics Dashboard - Transitions of Care

Transitions of Care guidelines are linked to a Benchmark Statistics Dashboard, which displays statistical information regarding utilization of services by patients falling under that guideline. The dashboards are accessed by a link marked "Benchmark Statistics" at the top of the guideline page.

The dashboard presents a variety of measures that correspond to the companion guidelines, including use of acute hospital, inpatient rehabilitation facility, and skilled nursing facility; admission and readmission rates; and length of stay.

The data are available for Medicare and Commercial populations, and benchmarks are shown for the total data sample, and for 9 multistate regions of the United States.

For more detailed information on data sources, benchmarks, and methodology, please refer to the Methodological Notes tab within the dashboards.

Abbreviations

The Abbreviations resource lists common medical and care guidelines abbreviations and their corresponding definitions. This resource also provides definitions for units of measurement.

Care Management Tools

Care Management Tools provide additional information on managing particular elements of care. These tools aid case managers and other healthcare professionals in creating and managing the treatment plan, and they help educate the patient and caregiver about treatment. Included in this section are various tools and calculators that address patient monitoring and assessment, and management of specialized types of care. The Care Management Tools section also provides definitions of levels of care across the continuum.

Quality Measures

The Quality Measures section describes the accreditation standards and performance measures that are addressed in the guidelines. The quality measures are intended to highlight areas of focus for major national quality initiatives or performance measures of national organizations. They are presented for reference purposes only and are neither an assurance of compliance nor a prescribed measurement set. The selection of accreditation organization, quality initiatives, measurements, and data collection instruments is at the user's discretion.

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